Royal Devon University Healthcare NHS Foundation Trust

Patient Diary Non-Clinical Guideline for the Implementation Use and Follow up in the Intensive Care Unit- RDUH Northern Services	
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Monitor/Finance/Performance		Develop Acute Services	
CQC Fundamental Standards Regulations No:		Delivery of Care Closer to Home	
		Infection Control	
Other *(please specify)*:			
Note: This document has been assessed for any equality, diversity or human rights implications			

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1.0	07/04/2024	Ruth Wood / Helen Marshall-Hawley	*New document to meet NICE CG83, GPICS standards, ICS and SWCCN recommendations*

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KEY POINTS OF THIS PROCEDURAL DOCUMENT:

Patients recovering from a critical illness may experience physical and psychological problems post discharge from Intensive Care Unit (ICU) that may impair their recovery (Ullman et al. 2015). Patient diaries can be used to help patients understand and come to terms with what has happened to them whilst they have been critically-ill. The diary is designed to provide a record of factual events which has occurred throughout a patient's admission to the ICU so as to improve any gaps in memory. They provide a context for memories that exist and can help dispel inaccurate and delusional beliefs.

The National Institute of Clinical Excellence (NICE) recommends that rehabilitation following critical illness starts as soon as possible and acknowledges that patient diaries can help patients to understand their critical care stay (NICE, 2009; Backman and Walther, 2001; Roulin et al. 2007). Providing intensive care patients with a diary has proven effective in reducing post-traumatic stress (Jones et al.2010) as well as anxiety and depression (Knowles and Tarrier, 2009) which is prevalent in many ICU survivors.

This document serves as a guideline for staff working within the intensive care setting on the standards that are expected to be achieved.

1. INTRODUCTION

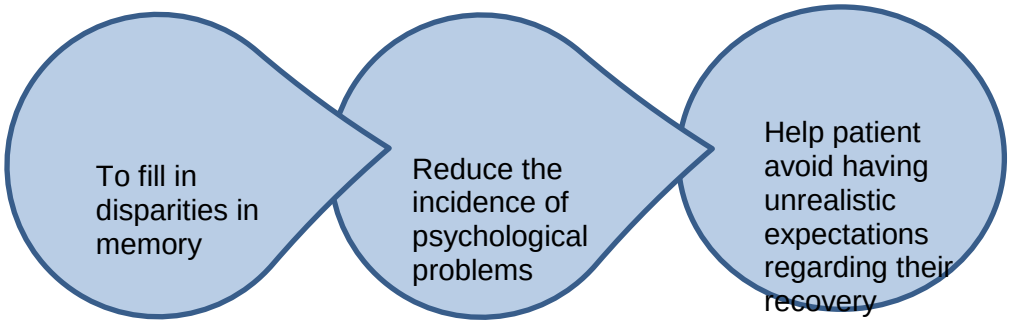
1.1. Patients recovering from a critical illness may experience physical and psychological problems post discharge from Intensive Care Unit (ICU) that may impair their recovery (Ullman *et al.* 2015). NICE guideline CG83 concludes that the use of a patient diary / journal can have beneficial impacts and reduce the psychological burden of a critical care stay for both patients and their relatives. Patient diaries can be used to help patients understand and come to terms with what has happened to them whilst they have been critically ill. The diary is designed to provide a record of factual events which has occurred throughout a patient's admission to the ICU so as to improve any gaps in memory. They provide a context for memories that exist and can help dispel inaccurate and delusional beliefs.

1.2. The National Institute of Clinical Excellence (NICE) recommends that rehabilitation following critical illness starts as soon as possible and acknowledges that patient diaries can help patients to understand their critical care stay (NICE, 2009; Backman and Walther, 2001; Roulin *et al*. 2007). Providing intensive care patients with a diary has proven effective in reducing post-traumatic stress (Jones *et al*.2010) as well as anxiety and depression (Knowles and Tarrier, 2009) which is prevalent in many ICU survivors.

2. PURPOSE AND SCOPE

2.1. This guideline is to enable critical care staff at Royal Devon and Exeter NHS trust, Northern site general intensive care unit to complete a diary which is appropriate to the needs of patients who have been admitted to critical care. It sets out how staff within the critical care team will be aware of the situations when a diary is to be commenced, the information that needs to be recorded within the diary and how the diary will be stored and shared with patients and/or relatives.
The guidance also sets out the requirements for storage retention and destruction of these diaries. This will ensure that all staff within Critical Care will be able to complete a diary which is appropriate to the needs of the patients who have been admitted to critical care.

2.2. The clinical aims of the diaries are:



3. DEFINITIONS

3.1. ICU – Intensive Care Unit

3.2. RDUH Northern site– Royal Devon University Trust, Northern site

3.3. NOK- Next of kin

3.4. AHP- Allied Health Professional

3.5. ACCP- Advanced Critical Care Practitioner

3.6. DART- Deterioration and Resuscitation Team

4. DUTIES AND RESPONSIBILITIES OF STAFF

4.1. The Clinical Matron for ICU is responsible for the circulation, awareness and implementation of this guideline.

The senior nursing team are responsible for ensuring the admin lead is aware of the diaries that have been commenced so the electronic system can be maintained.

The patient diary team is responsible for the evaluation of the guideline and its effectiveness, in addition to providing guidance and support to the ICU multidisciplinary team on its application.

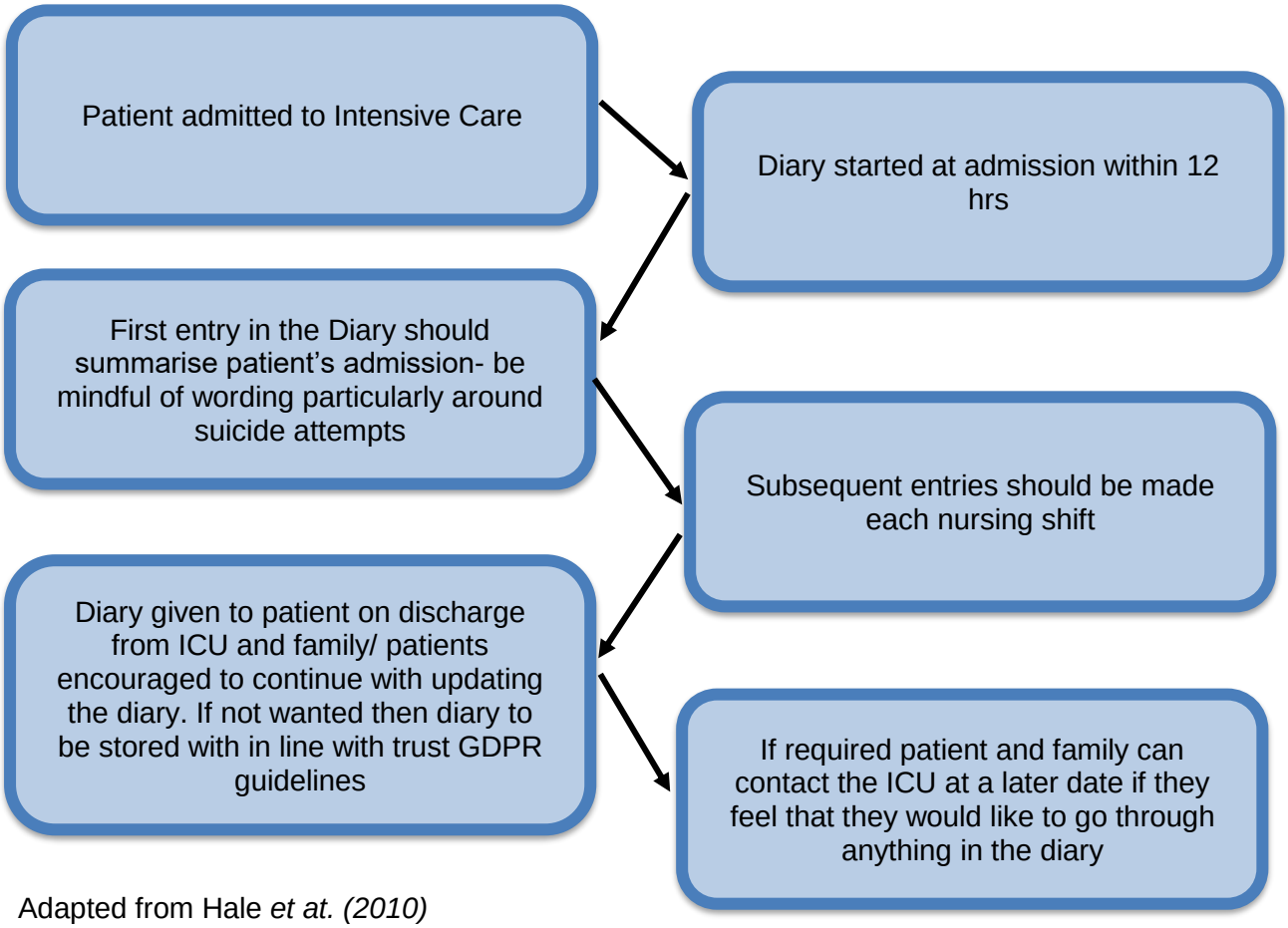
4.2. Clinical governance for ICU at RDUH is responsible for the review and ratification of this document and it's subsequent versions.

5. WHAT IS A PATIENT DIARY?

5.1. Diaries are written for patients in ICUs, usually by nurses and relatives (Bergbom *et al.* 1999; Backman and Walther, 2001). A patient diary is considered a way of keeping a very patient centred record of stay. Any member of the multidisciplinary team, patient's relatives and anyone that comes to visit the patient are encouraged to write in the diary using everyday language (Backman *et al.* 2014). An entry should be made at the beginning of the diary with the date and describing the reasons and details that led to the patient's admission (Zetterlund *et al.* 2012). Further entries that follow should include the patient's progress, observations, thoughts and reflections on the patient's condition (Backman *et al.* 2014).

6. PROCESS FLOW DIAGRAM OF THE IMPLEMENTATION AND FOLLOW-UP OF DIARIES

Figure 1



Adapted from Hale *et at. (2010)*

7. INCLUSION AND EXCLUSION CRITERIA

7.1. **Inclusion Criteria**

A diary should be considered for any patient admitted to the ICU. It is documented that all patients with at least 24 hours stay on ICU are likely to benefit from a diary, other patients may experience psychological distress and the benefit to them may be no less significant. Diaries should be started on admission to the ICU, explaining the process to the families giving them time to adjust to the ICU environment.

7.2. **Exclusion Criteria**

- Any patient who is admitted for organ donation
- Any elective patients (planned stay less than 36 hours) unless deemed to be appropriate.
- Patients under 18 years of age

8. MANAGEMENT OF DIARIES

8.1. All diaries are externally printed. The first few pages of the diary, there is space that should be completed with the patient's name, hospital number and the date the diary is commenced. No other patient identifiable information is required. There is also space to complete 'getting to know you' information. This information can be discovered from engaging with NOK about your patient.

8.2. New diaries will be kept in a dedicated drawer or wall holder on each unit and should be easily accessible to all staff so that they can be started promptly.

8.3. Admin staff are responsible for ordering new patient diaries and ensuring enough diaries are available.

8.4. When in use all diaries should be kept in top drawer of the patient's bedside storage trolley/ or designated place within the bedside.

8.5. When a diary is commenced, please ensure that the patients' name is added to the Diary Register (on ICU drive). This allows admin staff to keep track of who has a diary, and ensure it is given to patient or NOK on discharge of the patient from ICU. The register will contain the following information,

- Patient name
- Date diary commenced
- Date and location of patient discharge from ICU
- NOK details

8.6. The diary is the possession of the patient unless the patient passes away. In the event that their loved one dies or does not regain capacity the diary can be offered to the relative or loved-one. The relative or loved-one should be offered support through this process.

8.7. The diary should be given to the patient when they are discharged from the ICU. Families and patients should be encouraged to continue to write in the diary during their ICU stay.

9. DIARY CONTENT AND WRITING STYLE

9.1. Please use Black ink.

9.2. Entries should be made when there is a significant milestone or event to write about. If patient progress is slow still try to make regular entries. However, entries need not be made every shift.

9.3. A multi-disciplinary approach to the diaries is hoped for. All members of staff are invited and are welcome to make diary entries. A diary with contributions from nurses, doctors, AHP's, chaplains and relatives are likely to hold more meaning than a diary filled in by one person alone.

9.4. Avoid including information that could be of a sensitive nature or that a patient may wish to keep confidential (Malignancy, HIV, Substance abuse). A sensible approach is to write only what you would be comfortable to disclose verbally to a patient or relative at the bedside.
9.5. Avoid jargon and medical abbreviations. Use laymen's terms when describing clinical terminology. Try to relate what you write to how you would normally verbalise the information to a patient or relative.
9.6. Staff should sign the diaries using their first name only to maintain anonymity. No staff contact details should be put in the diary.
9.7. Writing style should always be professional and relevant, keep it factual. As much care and consideration should be taken with diary entries as any other form of professional documentation.
9.8. The diary is checked by each nurse on shift. If an entry was deemed not keeping to the above standards by the subsequent nurse, this would be referred to the nurse in charge / matron. The matron would consult the Psychologist for ICU for advice or any other members of the senior team as deemed appropriate. The page would be removed if it was deemed appropriate and this would be explained to the NOK, in line with duty of candour.
9.9. All entries should be dated. The first entry should include a description of the reason for ICU admission.
9.10. If you have any concerns about how to describe an aspect of care or an event for entry into a diary, please consult a member of the senior MDT team for advice.

10. EXAMPLES OF DIARY ENTRIES

Below are some examples of how it is expected to write in the diary. Please use everyday language. Entries should be made most days and should include information such as,

- Aims/ Plans for the shift
- Names of people who have visited
- Any changes in their condition
- Key Milestones reached
- Current events
- Any news from home

10.1. Some examples of how to write in the diary are shown below,

"You have shown some progress today, we have managed to reduce your oxygen requirements and you appear to be coping well. You are also requiring less assistance from the breathing machine. Your Mum and Dad have been sat at your bedside for the majority of the day"

"We have tried to reduce your sedation today but unfortunately the pressure in your head became too high so we have put you back to sleep and we will try again tomorrow. You have been visited by your friends this afternoon and they have been telling you about work"

"You have been quite unsettled today and unsure of your surroundings. The nursing team and your family have been giving you lots of reassurance"

"Today has been the first day you have sat out in chair, because you have been in bed for so long your muscles have become weak and so we have had to lift you out of bed using a hoist. However, you are having daily physiotherapy and you are getting stronger day by day"

Please do not write:

- Frustrations or profanity
- Inappropriate comments
- Information regarding family conflict
- Personal opinions

11. PATIENT SUPPORT

Patients may require support following reading their diaries and the opportunity to talk through their diary and experiences. This can be a very emotional and distressing experience for both the patient and their families. However, expressing emotions can be part of the recovery process and patients should be encouraged to talk about their experiences. If patients or relatives wish to talk about what has been written in their diary then let a member of the admin team know. The admin team can then refer the patient to either a member of the medical team or the Clinical Psychologist.

If further psychological support is required, then the patient will be highlighted to the clinical psychologist via email.

12. CONSENT

The patient is unlikely to be in the position to consent. Relatives will have the diary explained to them. If the relatives feel that they think that the diary would be inappropriate for their loved one, then they can refuse the diary from being written. The diary will remain the property of the patient. When the patient gains capacity then they will also be able to withdraw their consent from any further diary entries being made. In the event of this happening, the diary should be stopped and given to the patient if requested. The patient and relatives can also decline to receive the diary. In this case the diary should be kept within a secure locked cupboard on the unit. The diary should be kept for up to 10 years in line with GDPR guidelines.

13. END OF LIFE

In the event that the patient dies or does not regain capacity the diary can be offered to the relative or loved one. The relative or loved should be offered support with going through and reading the diary if required. If the family does not want the diary, the diary will be kept in a secure locked cupboard on the unit for up to 10 years.

14. ARCHIVING ARRANGEMENTS

The original version of this guideline, will remain with the authors Ruth Wood and Helen Marshall-Hawley. An electronic copy will be maintained on the Trust intranet, P

– Policies. Archived electronic copies will be stored on the Trust's "archived policies" shared drive, and will be held indefinitely. A paper copy (where one exists) will be retained for 10 years

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15. PROCESS FOR MONITORING COMPLIANCE WITH AND EFFECTIVENESS OF THE GUIDELINE

To evidence compliance with this policy, the following elements will be monitored:

What areas need to be monitored?	How will this be evidenced?	Where will this be reported and by whom?
Patients who have had a diary commenced, must be identified and a record kept on an electronic system	*The electronic system will be maintained by the admin lead for ICU and the senior nursing team and reviewed weekly.*	*The patient Diary team.* *ICU physiotherapy leads Ruth wood/Lisa Bashford.* *Clinical Matron for ICU* *Lead consultant for ICU*
Ensuring patient diaries are returned to patients and/or NOK, and in the circumstance that diaries are not wanted we are following correct GDPR guidelines for storage and safe keeping of diaries for 10 years	It will be evidenced on the electronic record maintained by lead admin for ICU and Diary team	Electronic record system maintained as per GDPR guidelines Also reported to the diary team

16. REFERENCES

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APPENDIX 1: COMMUNICATION PLAN

The following action plan will be enacted once the document has gone live.

Staff groups that need to have knowledge of the guideline	*Consultants* *All nurses working in ICU* *Static AHP's working in ICU* *ACCP's working in ICU* *Admin team working in ICU* *DART*
The key changes if a revised document	*NA*
The key objectives	*Formal processes and safe practice of patient diaries in ICU patients. Correct governance around patient diary's in ICU.*
How new staff will be made aware of the guideline and manager action	*Cascade by email from manager, induction process.* *Newsletter* *Team meetings* .
Specific Issues to be raised with staff	*Entire policy.*
Training available to staff	*Any member of the senior team.*
Any other requirements	
Issues following Equality Impact Assessment (if any)	*NA*
Location of hard / electronic copy of the document etc.	*:ICU/patientdiaries*